



CULTURAL SAFETY & GENDER-AFFIRMING CARE POLICY MANUAL

Respect, dignity, identity, and belonging for every person in care — SHSS D.1 and G.3.2(b)

Document Reference	ECC-CSGAC-POL-2026-001
Version / Effective Date	1.0 — July 2026
Approved By	Executive Director
Review Date	July 1, 2027 (annual)
Applies To	All staff, casuals, students, and volunteers at all ECC programs — MCFD (children & youth) and CLBC (adult) streams
Legislative / Standards Basis	MCFD SHSS D.1, G.3.2(b), B.1, E.3 (rev. April 2026); BC Human Rights Code (gender identity and expression, race, ancestry, religion protected); CFCSA (incl. s.70 rights and Indigenous identity provisions); An Act respecting First Nations, Inuit and Métis children, youth and families; Declaration on the Rights of Indigenous Peoples Act (BC); CLBC standards
Related Documents	ECC-RC-POL-2026-003; ECC-STAFF-BND-2026-001; ECC-BSP-2026-001; ECC-SHSS-DUTY-2026-001; ECC-POL-REG-2026-001

1. Policy Statement

Every child, youth, and adult in Evergreen Community Care's programs has the right to be cared for in an environment of respect and dignity that supports who they are — their culture, ancestry, language, religion or spirituality, Indigenous identity and community, gender identity, gender expression, and sexual orientation (SHSS D.1). Cultural safety and gender-affirming care are not optional kindnesses: they are mandatory practice standards, conditions of employment, and, under the BC Human Rights Code, legal obligations. Discrimination, ridicule, misgendering as a practice, denial of cultural or spiritual expression, or pressure to conform to any identity is prohibited conduct and is treated under the discipline and prohibited-practices provisions of the Behaviour Support Manual and HR Policy Binder.

Gender-affirming care training is mandatory for all staff (SHSS G.3.2(b)) — completed at orientation before working with persons in care and refreshed annually.

2. Definitions

Cultural safety	An outcome defined by the person receiving care: they experience the environment as safe for their identity, culture, and way of being. Staff create the conditions; the person in care judges whether it is safe
Cultural humility	A lifelong practice of self-reflection about one's own assumptions and biases, and learning from the person as the expert in their own culture and identity
Indigenous cultural safety	Care that acknowledges the history and ongoing impacts of colonization, residential schools, and child-welfare removals on First Nations, Métis, and Inuit peoples, and actively supports connection to family, community, land, language, and culture
Gender identity / expression	A person's internal sense of their gender, and how they present it (name, pronouns, clothing, hair, mannerisms). Protected grounds under the BC Human Rights Code
Gender-affirming care	Care that recognizes and supports a person's gender identity and expression — social affirmation (name, pronouns, expression, privacy) always; and support to access health care through the proper consent and clinical pathways where the person seeks it
2SLGBTQIA+	Two-Spirit, lesbian, gay, bisexual, transgender, queer/questioning, intersex, asexual, and additional identities. Two-Spirit is a distinct Indigenous identity and belongs in Indigenous cultural context
SOGIE	Sexual orientation, gender identity, and expression

3. Cultural Safety — Practice Standards

3.1 For every person in care

Learn at admission	Culture, language(s), religion/spirituality, foods, celebrations, community connections, and what respect looks like to them and their family — recorded in the care plan/ISP and reviewed at every care plan review
Food	Cultural and religious dietary needs are met in menu planning (halal, kosher, vegetarian, traditional foods where possible); food is never used to pressure conformity
Religion & spirituality	Persons in care choose their own religious/spiritual practice and participation; staff support attendance and observance and never impose their own beliefs (SHSS D.1; code of ethics)
Language	Support maintenance of first language; interpreters/translated materials arranged for the person and family where needed (SHSS B.1 — rights information in an understandable form)
Celebrations & belongings	Cultural and religious days recognized in the program calendar; cultural items and regalia are treated as significant personal property — handled per the privacy and belongings policy (SHSS B.3)
Anti-racism	Racist language, 'jokes', stereotyping, or differential treatment by staff, visitors, or peers is interrupted immediately, documented, and reported to the PC; incidents affecting a person in care are handled with them at the centre

3.2 Indigenous children, youth, and adults

Identity & belonging	An Indigenous person's connection to their family, community, Nation, language, land, and culture is a core need and right — planning proceeds with the Indigenous community per SHSS E.3 and any CFCSA s.92.1 / Nation agreements, coordinated through the social worker
Cultural planning	Each Indigenous person in care has cultural connection goals in their care plan/ISP: community contact, cultural events, ceremony, language, mentors/Elders — with named actions and staff responsibilities
Practice posture	Staff practice cultural humility: acknowledge the child-welfare system's history with Indigenous families; never disparage family or community; support (never obstruct) participation in ceremony and cultural practice, including smudging with appropriate safety arrangements
Two-Spirit	Two-Spirit identity is respected within its Indigenous cultural context; connect the person (with their consent) to Two-Spirit and Indigenous 2SLGBTQIA+ community resources
Community involvement	Invitations to the Indigenous community for planning and reviews are documented; barriers to participation are actively addressed (SHSS E.3)

4. Gender-Affirming Care — Practice Standards

4.1 Always — for every person

Names & pronouns	Use each person's chosen name and pronouns consistently — in speech, in the home, at school/community, and in day-to-day records (see 4.3). If you slip, correct yourself briefly and move on; repeated deliberate misgendering is a conduct issue
No outing	A person's gender identity or sexual orientation is theirs to share. Staff never disclose it — to family, school, peers, or other staff beyond need-to-know — without the person's consent, except where required by law or an imminent safety concern, and then with the SW involved. Being outed is a safety risk
Expression	Clothing, hair, grooming, and belongings that express the person's gender are supported, including in the clothing budget; there are no 'boys'/girls' rules for expression
Facilities & rooming	Bathroom and bedroom arrangements affirm the person's gender identity, planned with the person, PC, and SW, balancing everyone's privacy with dignity — the trans or non-binary person is never treated as the problem to be isolated
Peer conduct	Anti-2SLGBTQIA+ language or harassment by peers is addressed as a behaviour-support matter with education; the targeted person is supported, not moved or restricted as the 'solution'
Belonging	Positive representation in the home (books, media, community events like Pride where the person wishes); connection to 2SLGBTQIA+ youth/community groups offered, never forced

4.2 Health care access

Pathway	Where a person wants gender-affirming health care (assessment, counselling, or medical care), staff support access through the proper clinical and consent pathway: the SW/guardian (MCFD) or the adult and their supports (CLBC), the primary care provider, and BC's recognized referral routes (e.g., Trans Care BC resources)
Staff role	Staff facilitate appointments, provide transportation and support, and document — staff never make, delay, or block clinical decisions, and never substitute their own views for the clinical/consent process (mirrors ECC-MED prescriber-authority principle)
Consent	Health-care consent follows the Infants Act capacity framework for minors (with SW involvement per placement authority) and adult consent/substitute decision-making for CLBC; consent questions go to the PC → SW/prescriber, never resolved by front-line opinion
Mental health	2SLGBTQIA+ and Indigenous youth face elevated risks from discrimination and disconnection — affirmation is protective. Signs of distress are handled per the care plan and D.6 timely-health-care duty; concerns raised to PC/SW promptly

4.3 Records and documentation

Chosen name in daily records	Daily logs, room labels, schedules, and program-facing documents use the chosen name and pronouns; the legal name appears where legally required (MAR per pharmacy label, consents, court/ID documents) with the chosen name cross-referenced
Respectful documentation	Records never editorialize about a person's identity ('claims to be', 'phase') — document facts and the person's own words per the Documentation Standards
Confidential storage	Information about gender identity, transition, or orientation is need-to-know within the file per ECC-REC-POL-2026-001; discuss with the person what is written and shared where age/capacity-appropriate
ID documents	Where the person wishes to change name/gender markers, staff flag it to the SW (MCFD) or supports (CLBC) to action through legal channels

5. Prohibited Conduct

The following are prohibited practices under this policy, SHSS D.1/D.7, and the BC Human Rights Code, subject to discipline up to termination and, where applicable, reporting to the funder:

Identity suppression	Any attempt to change, discourage, ridicule, punish, or 'wait out' a person's gender identity, expression, or sexual orientation — including so-called conversion practices, which ECC prohibits absolutely
Deliberate misgendering / deadnaming	Persistent refusal to use chosen name and pronouns after correction
Outing	Disclosing a person's SOGIE or Indigenous/cultural information without consent outside need-to-know
Cultural denial	Preventing, mocking, or punishing cultural, spiritual, or religious practice; denying cultural foods or community contact as a consequence (also prohibited under D.7 — no arbitrary denial of family/community contact)
Proselytizing	Imposing staff religious or personal beliefs about culture, gender, or sexuality on persons in care
Discriminatory differential treatment	Applying different rules, chores, privileges, or tone based on race, culture, religion, or SOGIE

6. Responding to Breaches and Concerns

Person in care	Supported to use the complaint procedure (SHSS B.2) — including to the SW, RCY (children/youth), or Advocate; no reprisal ever; complaint documented and reported to the PC same day
Staff duty	Any staff member who witnesses prohibited conduct interrupts it, ensures the person is safe and supported, documents, and reports to the PC same shift (to the PM/ED if the PC is involved)
Escalation	Breaches affecting a person in care may be reportable incidents (SHSS C.2.8 — e.g., marked behaviour change, safety circumstance) — PC assesses same shift with the PM; SW notified where the standard applies
Pattern review	PM reviews cultural-safety and SOGIE-related complaints/incidents quarterly with behaviour records; systemic fixes go to the corrective action log

7. Training Requirements (SHSS G.3.2(b))

Training	Who	When	Renewal
Gender-affirming care training (mandatory)	All staff, casuals, students, volunteers	At orientation, BEFORE working with persons in care	Annually
Indigenous cultural safety training (e.g., recognized ICS course)	All staff	Within 6 months of hire	Refresher every 2 years
This policy — read and acknowledged	All staff	At orientation	On each annual re-issue
Program-specific cultural plan briefing (per person in care)	All program staff	At admission and each care plan review	Per review

Training completion is tracked with MALM and first aid on the monthly staff-currency check (by the 5th) and is a condition of continued scheduling.

8. Roles and Responsibilities

All staff	Practice the standards in Sections 3–4 every shift; interrupt and report breaches; complete mandatory training on time
PC	Builds cultural and identity information into admission and care plans; arranges interpreters, cultural foods, community connections; monitors daily practice; addresses peer conduct; reports breaches
PM	Ensures 100% training compliance (G.3.2(b)); investigates breaches; quarterly pattern review; liaises with SW/Indigenous community on planning; owns corrective actions
ED	Approves this policy annually; resources training and cultural supports; handles Human Rights Code complaints with counsel; reports to the Board/funder as required

9. Compliance Calendar

Task	DUE DATE	DATE DONE	INITIALS
Gender-affirming care training — new staff before first shift	Per hire		
Training currency check (with MALM/first aid)	Monthly, by the 5th		
Cultural connection goals reviewed in each care plan	Each review (≥ every 6 months)		
Quarterly pattern review — complaints/incidents	Quarter-end		
Annual gender-affirming refresher for all staff	Annually per staff		
Indigenous cultural safety training refreshers	Every 2 years per staff		
Policy review and ED re-approval	July 1, 2027		

Approval & Staff Acknowledgement

I have read and understood the Cultural Safety & Gender-Affirming Care Policy Manual (ECC-CSGAC-POL-2026-001). I understand the practice standards, the prohibited conduct, and that gender-affirming care training is mandatory before I work with persons in care and annually thereafter.

Executive Director — Signature / Date	
Program Manager — Signature / Date	
Staff Name (print) / Signature / Date	
Gender-affirming training completed (date)	
Indigenous cultural safety training (date)	